# COVID-19 Healthcare Funding Decision Framework

## Supplement to the *COVID-19 Healthcare Resource Roadmap (Version 2.0)*

This *COVID-19 Healthcare Funding Decision Framework* is intended to assist stakeholders who are navigating healthcare challenges attributable to the COVID-19 pandemic, including the identification, sequencing, administration, and management of funding resources. This document is a supplement to the *COVID-19 Healthcare Resource Roadmap (Version 2.0)* and has three main components: strategy identification, additional considerations for federal funding implementation and potential alignment, and case studies. The information that follows highlights how different funding streams can be applied to address one challenge area from the Roadmap: Vaccine Administration. However, the decision-making pathways and additional considerations outlined here may also be applied to other stakeholder challenges.

**Strategy Identification:** This section contains a visualization of the decision-making process that can be used when identifying potential funding to address the Vaccine Administration challenge. Users must answer:

- Which potential solution for Vaccine Administration needs support? Potential solutions highlighted in this document include transportation of the vaccine, establishment of temporary facilities for immunization, expansion/modification of storage space for proper vaccine storage, and administering immunizations (including labor costs, Personal Protective Equipment, and other supplies).
- What entity needs support? For purposes of this example, stakeholders will be either a State, Local, Tribal, or Territorial (SLTT) public health department, or a healthcare provider (such as a hospital, clinic, or other healthcare entity). By following the branches for each of these two questions, users will see a list of potential funding programs applicable to the identified strategy and the entity type. Each program is hyperlinked with the program webpage, and letters located in parentheses after the program name provide additional information on the funding type and targeted populations, if applicable.

**Targeted Populations Term Key**

- Direct Payment (D)
- Reimbursement (R)
- Veteran (V)
- Tribe (T)
- Senior (S)
- All (A)
- Rural (RR)

**Additional Considerations:** This section provides additional considerations stakeholders should take into account to adequately identify, sequence, administer, and manage federal funding programs applicable to COVID-19 response and recovery efforts. Technical assistance resources that may help with strategy development and project scoping are available in the *COVID-19 Healthcare Resource Roadmap (Version 2.0)*.

**Case Studies:** The final section provides case studies to help illustrate how multiple funding programs can be applied to address the Vaccine Administration challenge area. Case Study 1 focuses on a public health department in a rural state, while Case Study 2 focuses on a healthcare clinic in a suburb outside a major city. Note: these examples are for illustrative purposes only and should not be interpreted as a guidance or guarantee of funding for similar entities.

**Please note that this document is for general guidance purposes only and has been compiled with publicly available information or with information provided by sources that are publicly obtained. This should be viewed as only a starting point for navigating challenges and available resources. Additionally, the resources highlighted are available as of the date captured in the footer of the document and will be updated, as appropriate. The user should always directly consult the respective department or agency administering the federal program as the provider of a potential resource for current program information, as well as to verify whether proposed activities would be allowable under a particular program. Specific information on applicant eligibility, legal responsibility, cost share, and other program requirements should also be confirmed with the provider of that program.**

| Question 1: Which potential solution needs support? | Question 2: What entity needs support? | Available Programs (Term Key on Previous Page) | |
|---|---|---|---|
| **Transportation of the vaccine** | **SLTT** | CDC COVID-19 Public Health Crisis Response COVID-19 Funding (D, A)<br>CDC Immunization Cooperative Agreement (D, A)<br>CDC Public Health Emergency Preparedness (PHEP) Cooperative Agreement (D, A)<br>CDC Supporting Tribal Public Health Capacity in Coronavirus Preparedness and Response (D, T)<br>CDC Tribal Public Health Capacity Building and Quality Improvement (D, T) | FEMA 2020 Emergency Management Performance Grant Program COVID Supp (D, A)<br>FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>HHS Indian Health Service COVID 19 (D, T)<br>Treasury Coronavirus Relief Fund (D, A)<br>Treasury Coronavirus State and Local Fiscal Recovery Fund (R, A)<br>USDA Community Facilities Loan and Grants (R, RR) |
| | **Healthcare Providers** | FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>HHS-HRSA Health Center Program (D, A) | HRSA Provider Relief Fund (R, A)<br>USDA Community Facilities Loan and Grants (R, RR) |
| **Temporary facilities for immunization** | **SLTT** | CDC Supporting Tribal Public Health Capacity in Coronavirus Preparedness and Response (D, T)<br>CDC Tribal Public Health Capacity Building and Quality Improvement (D, T)<br>CDC Immunization Cooperative Agreement (D, A)<br>CDC Public Health Emergency Preparedness (PHEP) Cooperative Agreement (D, A)<br>CDC COVID-19 Public Health Crisis Response COVID-19 Funding (D, A)<br>FEMA 2020 Emergency Management Performance Grant Program COVID Supp (D, A)<br>FEMA Public Assistance – Category B Emergency Protective Measures (D, A) | HHS Indian Health Service COVID-19 (D, T)<br>HUD Community Development Block Grant (CDBG)-CV (R, A)<br>HUD Indian Community Block Grant (ICBG) (R, A)<br>Treasury Coronavirus Relief Fund (D, A)<br>Treasury Coronavirus State and Local Fiscal Recovery Fund (R, A)<br>USDA Rural Economic Development Loans and Grants (R, D)<br>USDA Community Facilities Loan and Grants (D, RR) |
| | **Healthcare Providers** | FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>HRSA Provider Relief Fund (R, A)<br>HHS-HRSA Health Center Program (D, A) | USDA Rural Economic Development Loans and Grants (D, RR)<br>USDA Community Facilities Loan and Grants (R, RR) |
| **Expansion/modification of storage space for proper vaccine storage** | **SLTT** | CDC Supporting Tribal Public Health Capacity in Coronavirus Preparedness and Response (D, T)<br>CDC Tribal Public Health Capacity Building and Quality Improvement (D, T)<br>CDC Immunization Cooperative Agreement (D, A)<br>CDC COVID-19 Public Health Crisis Response COVID-19 Funding (D, A)<br>CDC Public Health Emergency Preparedness (PHEP) Cooperative Agreement (D, A)<br>FEMA 2020 Emergency Management Performance Grant Program COVID Supp (D, A) | FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>HHS Indian Health Service COVID-19 (D, T)<br>HUD Community Development Block Grant (CDBG)-CV (R, A)<br>HUD Indian Community Block Grant (ICBG) (R, A)<br>Treasury Coronavirus Relief Fund (D, A)<br>Treasury Coronavirus State and Local Fiscal Recovery Fund (R, A)<br>USDA Community Facilities Loan and Grants (D, RR) |
| | **Healthcare Providers** | HHS-HRSA Health Center Program (D, A)<br>HRSA Provider Relief Fund (R, A) | FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>USDA Community Facilities Loan and Grants (D, RR) |
| **Administration of immunizations (e.g., labor costs, Personal Protective Equipment (PPE), and other supplies)** | **SLTT** | CDC Tribal Public Health Capacity Building and Quality Improvement (D, T)<br>CDC Supporting Tribal Public Health Capacity in Coronavirus Preparedness and Response (D, T)<br>CDC COVID-19 Public Health Crisis Response COVID-19 Funding (D, A)<br>CDC Public Health Emergency Preparedness (PHEP) Cooperative Agreement (D, A)<br>CDC Immunization Cooperative Agreement (D, A)<br>FEMA Public Assistance – Category B Emergency Protective Measures (D, A) | FEMA 2020 Emergency Management Performance Grant Program COVID Supp (D, A)<br>HHS Indian Health Service COVID-19 (D, T)<br>HUD Community Development Block Grant (CDBG)-CV (R, A)<br>HUD Indian Community Block Grant (ICBG) (R, A)<br>Treasury Coronavirus Relief Fund (D, A)<br>Treasury Coronavirus State and Local Fiscal Recovery Fund (R, A) |
| | **Healthcare Providers** | CMS Medicare Vaccines and Immunizations (R, A)<br>CMS Suspension of Medicare Sequestration (R, A)<br>FEMA Public Assistance – Category B Emergency Protective Measures (D, A)<br>HHS-HRSA Health Center Program (D, A) | HRSA Provider Relief Fund (R, A)<br>HRSA COVID-19 Claims Reimbursement for Healthcare Providers for Testing, Treatment, and Vaccine Administration for Uninsured (R, A) |

# Additional Considerations

Vaccine implementation requires stakeholders to plan every step of the process, from transportation and distribution to outreach and administration. Planning continues for the implementation of the immunization effort now that vaccines have been approved and distributed; however, requirements associated with delivery may differ by vaccine. Specific considerations for public health departments and healthcare providers include the type of storage that may be necessary, the rate at which the vaccine is available for use, and community priorities that may impact the distribution of the vaccine. To adequately address these considerations and others, stakeholders should address the following questions.

**Tracking and Reporting Information**

- Have I been tracking my expenses in a way that separates or signals expenses related to coronavirus to reduce duplication?
- Do I have access to a dashboard that tracks the financial impacts of the COVID-19 pandemic on a daily basis in order to submit monthly management reports or other documentation? If not, can I create one?
- Do I understand and can I consistently meet all reporting requirements of funding received?

**Funding**

- Do I have access to matching funds when and if required by the funder?
- Do I have private insurance or a financial reserve that can act as the first set of funding support?
- Have I reviewed available guidance for State Medicaid Programs (Coverage and Reimbursement of COVID-19 Vaccines, Vaccine Administration, and Cost-Sharing under Medicaid, the Children's Health Insurance Program, and Basic Health Program) and do I understand how to apply for coverage?
- When is the funding released, and what are the associated deadlines?
- Is it a direct payment or reimbursement?

**FEMA Eligibility**

- Am I an eligible applicant under the FEMA PA Program? Is the identified need an eligible use of FEMA PA Program funds?
- Is the work required a direct result of the declared event (i.e., COVID-19 pandemic)?
- Are the claimed costs both necessary and reasonable?

**Staffing Capacity**

- Which staff members can I designate to oversee administration and tracking of federal funds, insurance, and other funding streams and what training do they need?
- Which staff members and sites can I designate for implementation of vaccination plans?
- Which additional staff pools can I credential and train if needed for surge capacity (i.e., pharmacy, dental, medical, behavioral, other)?

**Communication**

- Have you reviewed available guidance from the CDC on communications resources for COVID-19? (Communication Resources for COVID-19 Vaccines)
- How do I identify and engage with underserved communities or populations with unique needs?
- How do I build and maintain community confidence in COVID-19 vaccines?

Note: the list of questions is not exhaustive and should be treated as a starting point for strategy development for specific solutions associated with vaccine administration. Stakeholders should consider consulting technical assistance resources when answers to the above questions are unclear. In addition to the *COVID-19 Healthcare Resource Roadmap (Version 2.0)*, ASPR TRACIE (HHS) has developed a COVID-19 Resource Library that contains technical assistance resources relevant to this challenge.

# Case Study 1: Rural County Public Health Department

The public health department for a rural county is preparing to receive shipments of two different vaccines and is seeking financial support for **expansion/modification of storage space for proper vaccine storage** and **transportation of the vaccine** ahead of distribution to qualified healthcare providers. After carefully reviewing the financial and reporting requirements for each program, the county decided to apply for the programs described below to address vaccination administration needs.

*Note: Federal funding programs require a level of administrative capacity to apply for and administer. You should determine your first tranche of funding based on your capacity to administer program requirements. This case study is one example of how funding can be applied to meet identified needs, and is for illustrative purposes only. This should not be interpreted as a guidance or guarantee of funding for similar entities.*

The county already owns a medical storage facility but needs to improve the facility to ensure it meets all requirements for proper storage of COVID-19 vaccines.

The US Treasury Coronavirus Relief Fund can provide funding to support the improvement of this facility. Expenses must be incurred by December 31, 2021.

*Direct Payment*

*Cost Share (Federal 100%)*

*Can Apply as Matching Funds*

The county needs to establish a medical transportation system for movement of COVID-19 vaccines from the storage facility to qualified healthcare providers.

The Public Health Emergency Preparedness (PHEP) Cooperative Agreement can be used to help manage, track, and distribute medical materiel.

*Direct Payment*

The county needs support for mobile COVID-19 vaccination in remote areas where access is limited. Needs include equipment and supplies for storage, handling, and transport to mobile vaccination centers.

The FEMA PA Program – Category B: Emergency Protective Measures can be applied to support mobile vaccination centers when the activity is not being funded by another federal department or agency (e.g., CDC). Note: The county would be a subrecipient of the state for PA.

*Direct Payment*

*Cost Share (Federal 100%)*

*Requires FEMA-State Agreement*

# Case Study 2: Local Non-Profit Healthcare Clinic

A non-profit healthcare clinic located in a small, older suburb just outside of a major city is preparing for vaccine administration efforts. The clinic serves a largely low-income client base utilizing Medicare/Medicaid. The clinic is seeking reimbursement for the **establishment of a temporary facility for immunization** and Personal Protective Equipment (PPE) purchases for staff **administering immunizations.** In addition to vaccination efforts, the clinic has to maintain its normal operations to continue serving day-to-day healthcare needs for the local population. After carefully reviewing the financial and reporting requirements for each program, the clinic decided to apply for the programs described below to address vaccination administration needs.

*Note: Federal funding programs require a level of administrative capacity to apply for and administer. You should determine your first tranche of funding based on your capacity to administer program requirements. This case study is one example of how funding can be applied to meet identified needs, and is for illustrative purposes only. This should not be interpreted as a guarantee of funding for similar entities.*

The clinic needs to establish a temporary facility for immunization, as the existing clinic space is not large enough for both vaccination efforts and day-to-day patient care.

The HHS HRSA Provider Relief Fund can be applied to reimburse the costs of establishing of a temporary facility. This program's funding is broadly applicable to the COVID-19-related needs of qualified healthcare providers.

*Reimbursement*

*Quarterly Reporting Required*

The clinic needs funding support for the administration of vaccines to its Medicare/Medicaid enrolled patient caseload.

The HHS CMS Medicare and Medicaid programs will provide reimbursement at a pre-determined payment rate for administration of vaccines to patients enrolled in Medicare or Medicaid. Since the current supply of COVID-19 vaccinations has been federally purchased, reimbursement is only applicable for vaccination administration at this time.

*Reimbursement*

*Pre-Determined Payment Rates*

The clinic also needs funding support for the administration of vaccines to and daily medical treatment needs of uninsured walk-in patients.

The HHS HRSA COVID-19 Claims Reimbursement to Health Care Providers for Testing, Treatment, and Vaccine Administration for the Uninsured Program can provide reimbursement both for vaccination of uninsured patients and day-to-day healthcare costs associated with testing and treating uninsured patients for COVID-19. Reimbursement under this program will generally be provided at pre-determined Medicare payment rates.

*Reimbursement*

*Pre-Determined Payment Rates*